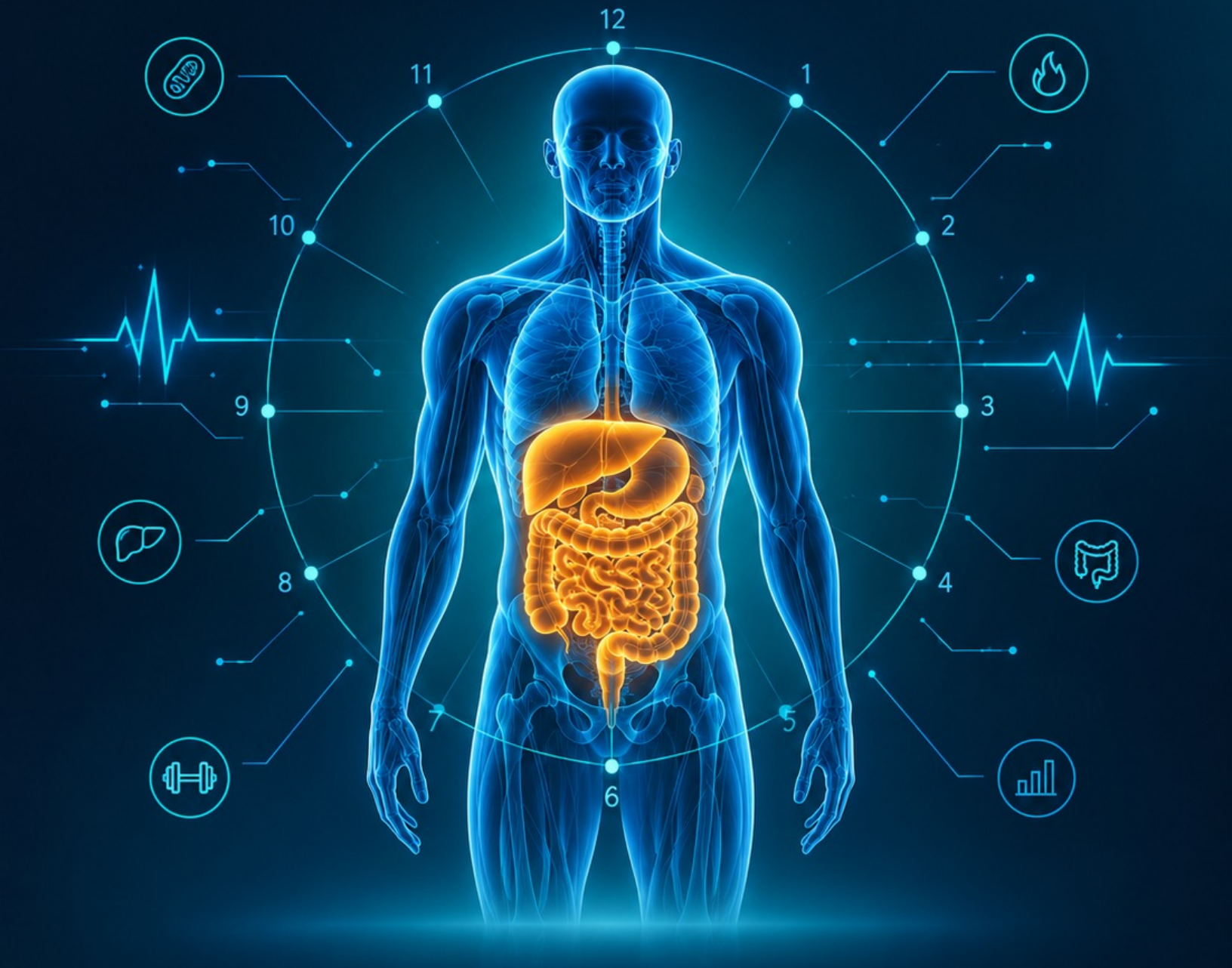
CLINICAL LONGEVITY SERIES

The 12-Week Metabolic Re-Engineering & Visceral Fat Protocol

Strip organ fat. Protect your muscle.



J. C. STORGANAU

The 12-Week Metabolic Re-Engineering & Visceral Fat Protocol

A simple, phased plan to reduce hidden belly fat, improve blood sugar control, and protect your muscle.

12	3	4	1
WEEKS	PHASES	DATA SOURCES	GP REVIEW

THE BIG IDEA: LOOKING HEALTHY ISN'T THE SAME AS BEING HEALTHY

You can have a normal Body Mass Index (BMI) and a normal scale weight and still carry a lot of fat around your organs. This is called **TOFI: Thin Outside, Fat Inside**. A bathroom scale only measures total weight. It can't tell muscle, water, and hidden organ fat apart, so serious metabolic problems can go unnoticed.

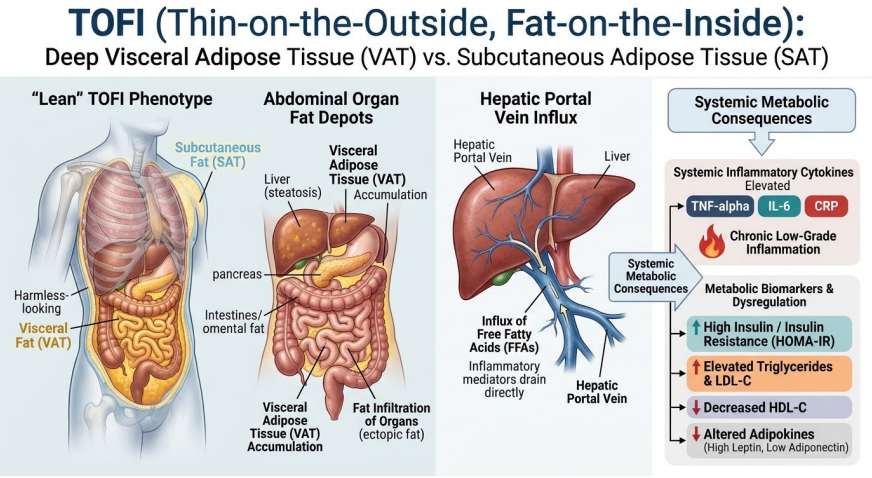


Figure 1: Fat under the skin (subcutaneous) vs. visceral fat, which drains straight into the liver.

Why hidden fat matters

- **Visceral fat** sits deep in the abdomen, around your organs. It isn't just storage; it acts like an active, inflammatory organ.
- It releases inflammatory signals (**TNF-α, IL-6**) straight to the liver. This helps fat build up in the liver and raises **hs-CRP**, a blood marker of inflammation.

What it does to your arteries

- A fatty liver makes your body respond less to insulin, which pushes up blood sugar (**HbA1c**).
- The liver also sends out more fat-carrying particles. **ApoB** counts them, and a higher count means higher artery-clogging risk.

The goal of this protocol

Your muscles clear most of the sugar from your blood after you eat (up to about 80% of insulin-driven uptake). Crash diets burn muscle along with fat, which slows your metabolism and worsens blood sugar control. This plan targets organ fat **while protecting your muscle**.

SECTION 1

Track Your Progress: The 4 Data Sources

One scale reading can mislead you. Instead, you'll collect data from four sources over time, then confirm the trend with blood tests.

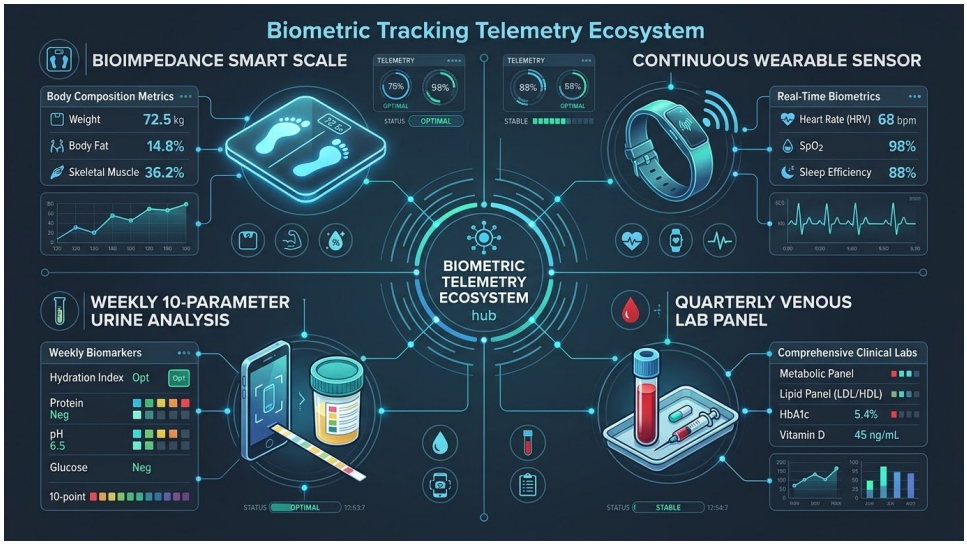


Figure 2: Your four tracking tools: smart scale, wearable, urine strips, and lab tests.

1. Smart scale (daily, morning)

- **Visceral fat rating:** you want this to trend down. On a 1-12 scale, 10+ is high and 1-5 is the ideal zone. Scales differ, so follow *your* scale's trend.
- **Skeletal muscle mass:** should hold steady or rise. If it drops, you're losing muscle.
- **Body fat %:** gives context to changes in weight.

2. Wearable: watch or ring (daily)

- **Nocturnal HRV (rMSSD):** a gauge of how well your nervous system recovers overnight. Higher is generally better, but compare against your own baseline.
- **Resting heart rate:** a steady or falling trend suggests better fitness and lower stress.
- **Deep sleep:** the stage where much of your physical repair happens.

3. Urine strip test (every Sunday)

- **Specific gravity (1.010-1.020):** shows hydration. Dehydration can skew smart-scale readings.
- **pH (typically 6.0-7.5):** reflects diet and hydration. Just log it; don't try to chase a number.
- **Ketones and protein:** ketones show you're burning fat for fuel. Persistent protein is worth mentioning to your GP.

4. Blood and urine labs (Week 0 and Week 12)

- **ApoB:** counts artery-clogging particles.
- **HbA1c:** your average blood sugar over about 90 days.
- **hs-CRP:** your whole-body inflammation level.
- **Urine ACR:** checks that your kidneys' filters are healthy.

Measurement rules for reliable data

1. Weigh yourself right after waking, after urinating, and before drinking water. **2.** Wear your sensor on the same wrist every day. **3.** Judge progress by the **7-day median** (the middle value of the week), never a single day. Home scales estimate body composition, so use them for trends, not as exact measurements.

SECTION 2

Your 12-Week Roadmap

Build easy habits first, add muscle-building second, and finish by reviewing your results with your doctor.

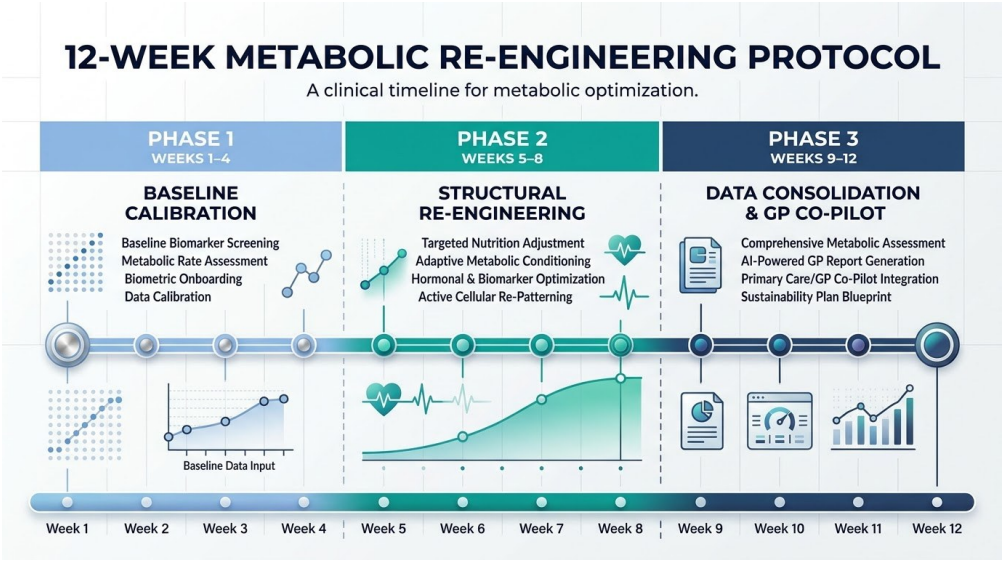


Figure 3: The three phases: Baseline Calibration, Structural Re-Engineering, and GP Co-Pilot.

PHASE 1 • WEEKS 1-4
Build the base
Collect baseline data and lock in three easy daily habits.

PHASE 2 • WEEKS 5-8
Build muscle
Add protein, fiber, strength training, and steady cardio.

PHASE 3 • WEEKS 9-12
Review & partner
Analyze your results and share them with your doctor.

PHASE 1 (WEEKS 1-4): THREE EASY DAILY HABITS

The first 28 days are about gathering clean baseline data and building habits that feel easy. There's no intense training yet. **Keep all three habits going for the full 12 weeks.**

1

10-Minute Walk After Meals

How: Within 15 minutes of lunch and dinner, take an easy walk at a pace where you can still hold a conversation.
Why it works: Working muscles pull sugar out of your blood without needing insulin (through GLUT4 transporters), which softens blood sugar spikes.

2

Morning Sunlight

How: Spend 10-15 minutes outdoors within 30 minutes of waking. Stay a bit longer on cloudy days.
Why it works: Morning light sets your body clock and shapes your daily cortisol (stress hormone) rhythm, which supports better sleep.

3

Midday Calm Reset

How: Between 1:00 and 3:00 PM, spend 10 minutes on Non-Sleep Deep Rest (a guided relaxation) or *physiological sighs* (a double inhale through the nose, then a long exhale through the mouth).
Why it works: It calms your stress response. Long-term high stress hormones are linked to storing more fat around the belly.

PHASE 2 • WEEKS 5-8

Build Muscle & Burn Organ Fat

Now that your habits are in place, you'll add the two things that protect and build muscle: enough protein and progressive strength training.

Protein: your muscle-building fuel

- Eat **1.6-2.2 g of protein per kg of body weight per day**, spread over 3-4 meals. Example: at 70 kg, that's about 110-155 g a day.
- Aim for roughly **30-40 g per meal**. That delivers the **2.5-3.5 g of leucine** (an amino acid) needed to switch on muscle building. Plant proteins may need slightly larger portions.

Fiber & food order

- Eat in this order: **protein, fiber, and fat first**, then starchy carbs. This blunts blood sugar spikes.
- Build up to **35-45 g of fiber a day** gradually, and drink plenty of water.
- Smart snacks: raw walnuts, Greek yogurt with chia seeds, or celery with hummus.

YOUR WEEKLY SCHEDULE

MON	TUE	WED	THU	FRI	SAT	SUN
Strength A	Zone 2 cardio	Strength B	Zone 2 cardio	Strength C	Easy walk or rest	Rest + urine test

Keep your three Phase 1 habits (walks, sunlight, calm reset) every day on top of this schedule.

STRENGTH WORKOUTS (3 DAYS A WEEK)

Do **3-4 sets of 8-12 reps** per exercise, resting 1-2 minutes between sets. Finish each set with **1-2 reps left in the tank** (this is called RIR 1-2). When you can complete every set at 12 reps with good form, add a little weight next time.

Mon: Squat & Push

- Goblet or barbell squat
- Push-ups or dumbbell bench
- Inverted bodyweight rows
- Front and side planks

Wed: Hinge & Pull

- Romanian deadlifts
- Single-arm dumbbell rows
- Lat pulldowns or pull-ups
- Glute bridge holds

Fri: Single-Leg & Core

- Walking dumbbell lunges
- Standing dumbbell overhead press
- Heavy farmer's carries
- Pallof press or dead bugs

Zone 2 cardio (Tuesday and Thursday)

Do **35-45 minutes** of steady cardio, such as incline walking or a stationary bike, at a pace where you can still talk in full sentences. Lab testing would put this at a blood lactate of 1.5-2.0 mmol/L, but the talk test is a reliable stand-in. This builds your cells' ability to burn fat for fuel.

Safety first: talk to your doctor before starting if you...

take medication for diabetes or blood pressure (changes in diet and exercise can change the dose you need); have kidney disease (higher protein intake may not be suitable); have a heart condition or new/unexplained symptoms; are pregnant or breastfeeding; have a history of disordered eating; or have an injury that affects training. Stop and seek medical advice if you feel chest pain, dizziness, or unusual shortness of breath.

PHASE 3 • WEEKS 9-12

Review Your Data & Partner With Your Doctor

The final phase turns four weeks of tracking into a clear picture you can share with your GP.

1	**Calculate your 14-day medians (Weeks 11-12)** For visceral fat, muscle mass, and HRV, take the middle value of your last 14 days. This smooths out day-to-day noise.
2	**Repeat your lab tests** Retake the blood panel (ApoB, HbA1c, hs-CRP) and the urine ACR. Book these around Week 11 so results are ready for your appointment.
3	**Generate your GP Brief** Go to **123TheNextLevel.com/clinician-copilot**, enter your numbers, and export the structured 2-page brief.
4	**Book your appointment** Hand your doctor the brief and the completed Worksheet 3 (the leave-behind summary on the last page).

THE 10-MINUTE GP APPOINTMENT

A short visit works best with a plan. Use this split:

0-2 min	2-6 min	6-10 min
Introduce your 12-week protocol	Review your lab changes (ApoB, hs-CRP, HbA1c)	Review medications and agree long-term targets

HOW TO READ YOUR RESULTS

Measure	What improvement looks like	What it means
Visceral fat rating	Lower than Week 0	Less fat around your organs
Skeletal muscle mass	Same or higher	Your muscle was protected
HRV and resting heart rate	HRV up or steady; heart rate down or steady	Better recovery and cardiovascular fitness
ApoB, hs-CRP, HbA1c	Lower or stable, moving toward targets	Lower vascular and metabolic risk (your GP interprets these)

Your scale weight may barely change while your body composition improves. That's expected and is exactly why you track more than weight.

SECTION 3

Printable Logging Worksheets

Record your numbers as you go. Print extra copies of Worksheet 1 (one per week) or keep them in a health binder.

WORKSHEET 1: DAILY SCALE & WEARABLE LOG (Week: _____)

Day	Time	Weight (kg)	Visceral (1-12)	Muscle (SMM)	Body fat %	HRV (ms)	RHR (bpm)	Walk: Lunch	Walk: Dinner
Mon	__:__							[ ]	[ ]
Tue	__:__							[ ]	[ ]
Wed	__:__							[ ]	[ ]
Thu	__:__							[ ]	[ ]
Fri	__:__							[ ]	[ ]
Sat	__:__							[ ]	[ ]
Sun	__:__							[ ]	[ ]
7-day median									

WORKSHEET 2: WEEKLY SUNDAY URINE LOG (Weeks 1-12)

Wk	Date	Specific gravity (goal 1.010-1.020)	Urine pH (typical 6.0-7.5)	Ketones (mg/dL)	Protein	Notes
01	__ / __					
02	__ / __					
03	__ / __					
04	__ / __					
05	__ / __					
06	__ / __					
07	__ / __					
08	__ / __					
09	__ / __					
10	__ / __					
11	__ / __					
12	__ / __					

Clean-catch method: collect the middle portion of your first morning urine, before eating or drinking. Read each strip pad at the exact time printed on the bottle's instructions.

WORKSHEET 3

Your GP Leave-Behind Summary

Fill this in before your Week 12 appointment and hand a copy to your doctor.

Patient name: _____ **Date of birth:** _____ **Date of review:** _____

1. LAB RESULTS: BEFORE VS. AFTER

Biomarker	Baseline (Wk 0)	Exit (Wk 12)	Common target	Result (circle one)
ApoB	____ mg/dL	____ mg/dL	Below 80 (optimal below 65)	Lowered / Not yet
HbA1c	____ %	____ %	Below 5.7%	Stable or lower / Higher
hs-CRP	____ mg/L	____ mg/L	Below 1.0 mg/L	Decreased / Not yet
Urine ACR	____ mg/g	____ mg/g	Below 30 mg/g	Preserved / Higher

2. BODY COMPOSITION & RECOVERY (14-DAY MEDIANS)

Metric	Baseline (Wk 0)	Exit (Wk 12)	Change	Question to answer
Visceral fat rating (1-12)	____	____	____ pts	Organ fat down?
Skeletal muscle mass	____ kg or %	____ kg or %	____	Muscle kept?
Nocturnal HRV (rMSSD)	____ ms	____ ms	____ ms	Recovery improved?
Resting heart rate	____ bpm	____ bpm	____ bpm	Heart fitness improved?

3. QUESTIONS FOR MY DOCTOR

1. Given my ApoB and hs-CRP changes, how do you assess my remaining cardiovascular risk?

2. Based on my updated blood sugar and visceral fat numbers, should any of my current medications be reviewed or adjusted?

3. How soon should I repeat my blood tests?

My own notes / questions: _____

_____

Get your kits and your automated GP report

Blood and urine testing kits (US, UK, and Spain): **123TheNextLevel.com**
Generate your GP report: **123TheNextLevel.com/clinician-copilot**

Disclaimer: This guide is for education and self-improvement only. It does not diagnose conditions or replace personal medical care. Always check with your GP or physician before changing your exercise, nutrition, or medications. Targets shown are common reference values; your doctor may set different ones for you.